

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: TMillerPeriod: July 3 to October 1, 2024Report Date: October 2, 2024Coverage: 1608/2184h (74%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context

Patient clinical status over 91 days from Jul 03 to Oct 01

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	60 bpm	48 bpm	75 bpm
Breathing	18 brpm	11 brpm	53 brpm

Last 24 hours: two elevated breathing episodes in the morning; one high breathing episode in the afternoon.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Oct 01	11 AM to 12 PM	3h	3	Elevated Breathing	27	13	53	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 01	12 PM to 1 PM	1h	1	High Breathing (brief)	32	22	47	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

70 episodic events Detected, spanning 4 days total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
70	108h	Low Heart Rate	<1	74%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 26	1 PM to 2 PM	3h	3	High Breathing	35	11	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 15	2 PM to 3 PM	2h	2	High Breathing	32	16	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 22	1 PM to 2 PM	2h	2	High Breathing	35	12	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Jul 31	9 PM to 11 PM	8h	3	Low Heart Rate	42	40	47	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 09	11 AM to 12 PM	24h	10	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 24	12 PM to 2 PM	28h	12	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

+ 5 additional condition(s) across Elevated Breathing; details in monitoring data.

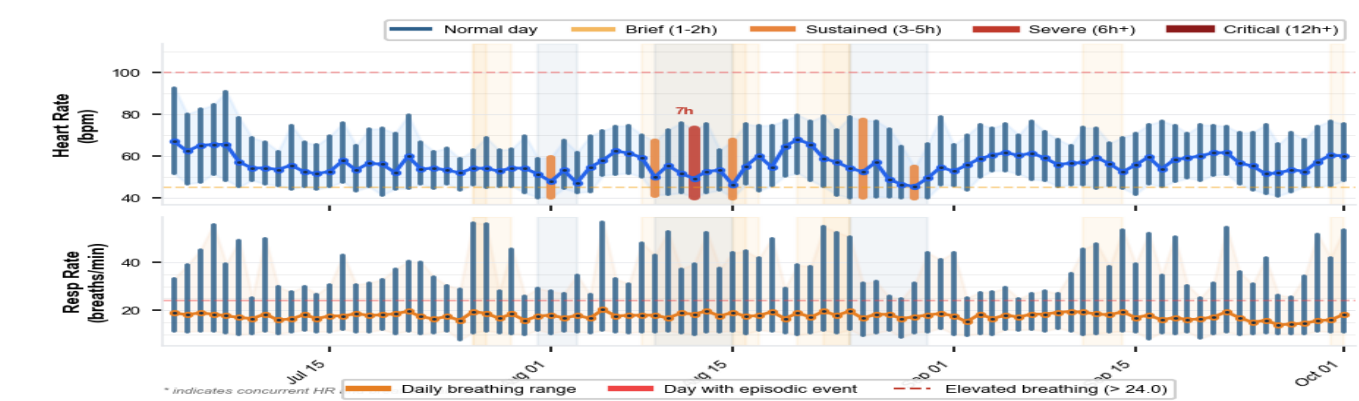
The P5 to P95 heart rate spread was 23 bpm (46 to 69), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 53 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 35 brpm, peak 56 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 11 unstable phases with 23 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

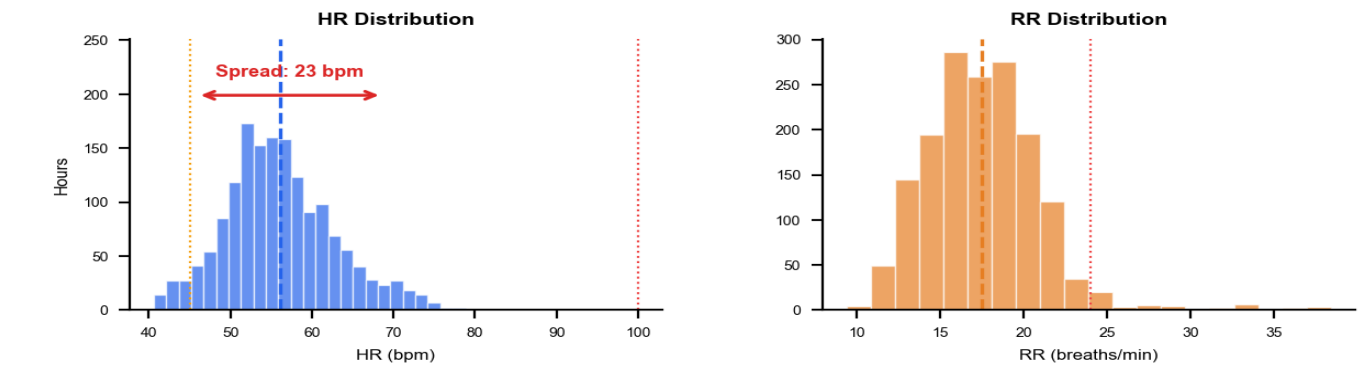
Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 23 bpm (45 to 68).
- 3. **Clustered pattern:** Episodic events on 6 of 91 days (6%).

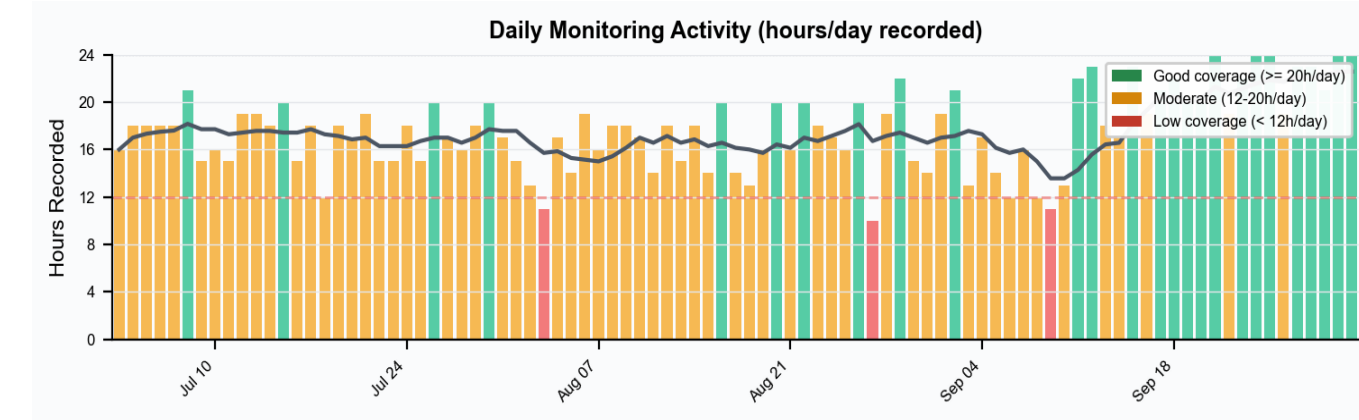


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 bpm
- High Breathing > 30 bpm
- Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.